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Louise Ings
Claims Manager

13-Feb-2025

RE: Report for Mr Cory Shepherd (11-Jan-1991)

**104/158 Scarborough Street
Southport QLD 4215 0422 438 627**

Many thanks for reaching out for the report. I have undertaken clinical assessment, reviewed materials presented to me regarding his unfair dismissal, workplace conflicts, alleged bullying, harassment and Workcover matter. Further obtained collateral information from his mother independently. Based on above, I am able to provide information on his mental health symptoms, working diagnosis and opinion on impact of his mental health on his daily functioning and occupational impairment. **Mr Cory Shepherd suffers from Major Depressive Disorder with anxious distress (DSM 5- 296.23).**

Mr Cory Shepherd presented with workplace stress stemming from issues with management and rostering at Queensland Health. He reported financial difficulties due to withheld pay, relationship strain, sleep disturbances, and mood changes with difficulty concentrating. These issues began approximately one year ago when a new manager was appointed. After raising concerns about rostering, fatigue management, and workplace policies, he experienced changes in work conditions including shorter breaks, more night shifts, and reduced penalty rates. His pay was withheld or delayed for up to five months at a time, leading to significant financial stress and strain on his relationship.

Cory presented as articulate and engaged during the consultation. Appeared physically drained and showed signs of neglecting personal hygiene, reporting a decrease in his usual self-care routine. He was anxious throughout the interview with intermittent intense sweating, shaking and hesitant speech. Subjectively he described mood as low, rating it 3/10. Affect was observed to be restricted with anxious distress. Expressed feelings of worthlessness and dehumanisation related to his current employment situation and financial stress. Thought Form and Content: Thought form was coherent and goal-directed. Thought content was preoccupied with workplace issues, including perceived unfair treatment, financial stress due to withheld pay, and the ongoing legal proceedings. Demonstrated rumination about these issues, which appeared to dominate his cognitive processes. There was no evidence of perceptual disturbances or hallucinations reported or observed during the consultation. He was oriented to time, place, and person, however was not able to sustain focus on conversation, kept repeating the same issues despite gentle nudging to relevant questions, evident of obsessive nature to his thoughts affecting his cognitive functions. He reported difficulties with concentration and literacy during periods of high stress at work. No formal cognitive assessment was conducted during this consultation. Cory demonstrated partial insight into his current mental state and the impact of workplace stress on his overall functioning. Judgement appeared impaired due to his mental state.

Risk Assessment: No current suicidal ideation or plan reported. Protective factors include a supportive partner and willingness to seek help. However, ongoing financial and occupational stressors may potentially increase risk in the future. There was no current suicidal ideation reported, and no evidence of perceptual disturbances or thought disorder. Insight was assessed as partial.

The severity of his symptoms has significantly impacted his occupational functioning, leading to his current unemployment. Social functioning has been impaired, particularly in his relationship, and he reported reduced self-care with a decreased hygiene routine. Cognitive difficulties, particularly with concentration and working memory.

The treatment plan includes continuing fluoxetine 20mg daily with monitoring for side effects and efficacy. Engagement in self-care activities and stress management techniques has been encouraged, and a referral for psychological therapy (CBT) is being considered. He started treatment with Fluoxetine which was increased to 3 capsules per day and night-time Quetiapine. Consideration was given to adding a low-dose antipsychotic-quetiapine 25mg nocte, for rumination and sleep disturbances.

1. Please detail the impact of the condition on ability to engage in daily activities (such as self-care, domestic activities, driving and community engagement).

Cory's condition has significantly impacted overall daily functioning, with notable effects on occupational performance, financial stability, personal relationships, and self-care routines. The stress and anxiety related to the workplace issues appear to have permeated multiple areas of Cory's life, leading to a marked decrease in his quality of life and ability to engage in normal daily activities.

Cory's workplace stress and related issues have significantly affected various aspects of his daily functioning:

- a. Occupational functioning: Currently not working due to ongoing workplace issues and disputes.
- b. financial management: Experiencing financial difficulties due to withheld or delayed pay, impacting overall quality of life.
- c. Relationships: Strain in current relationship due to financial stress, with partner feeling like a "sugar mommy".
- d. Self-care: Reported decline in general hygiene routines, no longer maintaining daily shower habits as previously done. He has been experiencing sleep disturbances, initial insomnia, obsessive rumination, fear driven thoughts, extensive researching, and planning on fact findings for work-related issues including reading up on policies, procedures, and legislations. His concentration was impaired, reported becoming "illiterate" due to stress, affecting ability to communicate effectively. He has been feeling flat in his mood, varied with unexplained irritability, self-directed anger, and feelings of worthlessness. He stopped spending time with his partner, friends and became socially withdrawn. He was unable to engage in previous interests and activities, suggesting a loss of enjoyment in previously pleasurable pursuits. He stopped exercising and has been neglecting his nutritional needs. His self-confidence and self-esteem had been low to an extent that he started to doubt his own values, beliefs, and ability to make his own decision.

2. Are there any other non-medical factors influencing capacity to work? If yes, please provide brief details.

Yes, there are several non-medical factors influencing capacity to work. Firstly, financial stress due to withheld or delayed pay for up to 5 months at a time has significantly impacted overall wellbeing and ability to meet basic needs. This has led to relationship strain and feelings of worthlessness. Ongoing workplace dispute and legal proceedings, including WorkCover claim and industrial relations review, have consumed his whole life severely affecting his functioning. The prolonged nature of these processes is contributing to rumination and preoccupation with workplace issues. His loss of routine and daily structure had changed since stopping work, disrupted normal coping mechanisms and self-care practices. This has led to a decline in personal hygiene habits and engagement in previously enjoyed activities. His perceived discrimination and unfair treatment in the workplace have eroded self-esteem and confidence in professional abilities. This may impact future job-seeking behaviours and workplace interactions. Cory had limited support from his union representatives has left feeling isolated and unsupported in navigating complex workplace issues, potentially exacerbating stress, and anxiety.

3. What is the current treatment plan and effectiveness of this treatment? Include type of

treatment, frequency, duration, starting/ending dates or ongoing (timeframes).

Current Treatment Plan:

Pharmacological Medications:

1. Fluoxetine 20mg daily, gradually increased to 3 in the morning, response has been gradual in nature.
2. Addition of low-dose antipsychotic -quetiapine 25mg nocte for rumination and sleep, helped him to switch off from the obsessive thoughts

3. Vyvanse 30 mg once a day started recently to manage his treatment resistant depressive state and cognitive impairment

Psychological treatment:

Cory's condition would benefit from regular psychological therapy (CBT) sessions and trauma focused interventions. I will be exploring his willingness to engage in therapy sessions.

Regular follow-up review scheduled to monitor progress, review mental state, and optimise treatment.

4. Please detail how the claimed condition impacts capacity to engage in the duties required of their own occupation:

a. Current capacity for work:

In my opinion Mr Shepherd has complete incapacity to undertake duties required for his own occupation.

b. Current barriers for returning to work within their own occupation:

Ongoing symptoms of severe nature of Major depressive Disorder and significant functional decline

c. Recommendations or support that can be made to assist in returning to work:

- Continue current medications review and optimisation required based on the progress
- Engage in psychological therapy (CBT) to manage symptoms and develop coping strategies
- Implement stress management techniques and self-care activities
- Regular psychiatric follow-up to monitor progress and adjust treatment as needed

d. Long-term barriers:

The duration of these barriers is unclear at this stage. Ongoing treatment and support may help address the current issues, but the outcome of the workplace dispute and legal proceedings could potentially have long-term impacts on the individual's ability to return to their previous occupation. Further, focusing on recovery is key to build psychological capacity to return to work with appropriate focused interventions to determine work-readiness.

5. Please briefly outline current Return to Work goal. If unclear, please provide an indication of a suitable time to review.

Given the complex workplace issues, ongoing WorkCover claim and disputed unfair dismissal proceedings and impaired decision-making abilities it is unclear at this stage the work goal and return to work. I will further update on this after next consultation. Please note that premature exposure to the workplace is more likely result in significant deterioration in his mental health and increased risks of suicide.

6. QSuper would like to partner with you to enable a safe and sustainable return to work, when medically suitable. QSuper can aid via Occupational Rehabilitation to help plan and facilitate suitable return to work goals. Occupational Rehabilitation can also provide strategies to help uplift work-readiness and provide additional support to Cory Shepherd through their return-to-work journey.

a. Is the engagement of an occupation rehabilitation provider likely to assist in facilitating a return to work in the next 1–2 months?

No, the engagement of an occupational rehabilitation provider is not likely to assist in facilitating a return to work in the next 1-2 months due to ongoing symptoms, impaired decision-making ability, and judgement.

b. If no, advise why and when you consider this assistance would be appropriate, or alternatively what strategies or support is being or could be utilised to enable a successful return to work?

The assistance of an occupational rehabilitation provider would not be appropriate at this time due to the following reasons:

1. Current mental health status: Cory is experiencing significant psychological distress due to multiple life stressors, including relationship breakdown, job loss, and bereavement.

2. Ongoing legal issues: Corey is involved in unfair dismissal proceedings with his employer, Queensland Health. This unresolved situation may complicate any return-to-work plans.

3. Need for stabilisation: The current focus is on stabilising Cory's mental health through medication management and psychological interventions

4. Recommendation to defer major decisions: I have advised Cory to defer in making decisions regarding work settlement or legal matters until his mental state improves.

Occupational rehabilitation assistance would be more appropriate once Cory's mental health has stabilised and the legal issues with his former employer have been resolved. This may take several months. Current strategies and support being utilised to enable a successful return to work in the future include:

1. Medication management: Restarting Vyvanse 30 mg daily to assist with focus and cognitive function, alongside continuing Fluoxetine for mood symptoms.

2. Psychological support: Ongoing counselling with Catherine, focusing on coping strategies and emotional regulation and focused CBT/trauma informed therapy

3. Lifestyle interventions: Implementation of a strict daily routine, including self-care activities and sleep hygiene.

4. Regular psychiatric review: A follow-up appointment is scheduled in 4 weeks to assess response to medication changes and routine implementation.

Once Corey's mental health has stabilised and legal issues are resolved, engagement with an occupational rehabilitation provider could be considered to facilitate a gradual return to work.

7. Do you have any additional comments to add not covered in the above?

Cory described himself as detail-oriented, perfectionistic, and having high standards. He tends to hyperfocus on tasks and can become consumed by issues he perceives as unjust. Cory's mother described him as clever, talented, and determined. She reported feeling undervalued and frustrated with perceived inconsistencies in policy implementation and described as having a strong work ethic. However, he struggles with assertiveness and decision-making, particularly in workplace and relationship contexts.

Please feel free to contact me on 0756545580 if you have any questions.

Yours sincerely,



Dr Ravikumar Bangalore Krishnaiah

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